

MEDICAL DISCHARGE SUMMARY (Sample)

Patient Name: John Perera

Age: 58 years

Sex: Male

Hospital Number: MRN-2026-104582

Date of Admission: 12 June 2026

Date of Discharge: 15 June 2026

Chief Complaint

The patient presented to the emergency department with intermittent central chest discomfort lasting approximately two hours. The pain was described as pressure-like, radiating to the left shoulder, and associated with mild shortness of breath and sweating. No episodes of loss of consciousness or palpitations were reported.

History of Present Illness

The patient has a past medical history of hypertension, type 2 diabetes mellitus, and hyperlipidemia. He reported that the chest discomfort began while climbing stairs and improved partially with rest. He denied fever, cough, abdominal pain, or recent trauma. Medication adherence was inconsistent during the previous month.

Past Medical History

- Hypertension (10 years)
- Type 2 Diabetes Mellitus (8 years)
- Hyperlipidemia
- No known chronic kidney disease
- No previous myocardial infarction

Allergies

No known drug allergies.

Examination on Admission

- Blood Pressure: 158/96 mmHg
- Heart Rate: 94 bpm
- Respiratory Rate: 18 breaths/min
- Temperature: 36.8°C
- Oxygen Saturation: 98% on room air

The patient was alert and oriented. Cardiovascular examination revealed normal heart sounds without murmurs. Chest examination demonstrated clear bilateral air entry. No peripheral edema was noted.

Investigations

Electrocardiogram showed nonspecific ST-segment changes without evidence of acute ST-elevation. Serial cardiac troponin levels remained within the normal reference range. Complete blood count was unremarkable. Serum creatinine was 0.9 mg/dL with an estimated glomerular filtration rate above 90 mL/min/1.73 m². Fasting blood glucose measured 162 mg/dL, and HbA1c was 7.8%. Lipid profile demonstrated elevated LDL cholesterol of 148 mg/dL.

Chest radiograph showed no acute cardiopulmonary abnormality. Transthoracic echocardiography demonstrated preserved left ventricular systolic function with an estimated ejection fraction of 60% and no significant valvular disease.

Hospital Course

The patient was admitted for observation and evaluation of possible acute coronary syndrome. Continuous cardiac monitoring remained stable throughout admission. Blood pressure and blood glucose were managed with medication adjustments. Cardiology consultation concluded that the presentation was most consistent with stable angina rather than acute myocardial infarction. Lifestyle modification and improved medication compliance were emphasized.

Discharge Medications

- Aspirin 81 mg orally once daily
- Atorvastatin 40 mg orally at night
- Metformin 1000 mg orally twice daily with meals
- Amlodipine 5 mg orally once daily
- Losartan 50 mg orally once daily
- Glyceryl trinitrate sublingual tablets as needed for chest pain

Discharge Diagnosis

1. Stable angina
2. Hypertension
3. Type 2 diabetes mellitus
4. Hyperlipidemia

Follow-up Recommendations

The patient should follow up with the cardiology clinic within two weeks and with the primary care physician within one week for blood pressure and diabetes review. Home blood pressure and blood glucose monitoring are recommended. A low-sodium, heart-healthy diet and regular moderate exercise are advised. The patient was instructed to seek immediate medical attention if chest pain becomes severe, persistent, occurs at rest, or is accompanied by shortness of breath, dizziness, or loss of consciousness.